

Medical Devices English

Instructor guide for advanced ESL learners working in medical devices

Audience: medical device engineers, product managers, quality and regulatory staff, clinical specialists, manufacturing teams, field-service teams, and device-company leaders

Focus: A device-focused professional English curriculum for design controls, risk management, usability, verification and validation, regulatory pathways, complaint handling, quality systems, manufacturing, and clinical-user dialogue.

Designed for advanced ESL learners who already use professional English and need industry-specific terminology, realistic meetings, role-play pressure, careful pushback, and polished workplace outputs.

Teaching stance: this is language and workplace-communication training, not legal, medical, financial, safety, or regulatory advice. Instructors should connect every scenario to the learner's current company policies, local rules, and approved procedures.

Purpose and Course Logic

A device-focused professional English curriculum for design controls, risk management, usability, verification and validation, regulatory pathways, complaint handling, quality systems, manufacturing, and clinical-user dialogue.

Core language challenge

Advanced learners do not only need vocabulary. They need the ability to ask which standard applies, who owns the decision, what evidence is sufficient, what risk is being accepted, and how to disagree without sounding vague, defensive, or reckless.

Each module trains a realistic workplace pressure point with role-specific terms, decision language, pushback practice, and bounded decision activities learners can apply to their own work.

Course objectives

- Use medical devices terminology accurately in meetings, written updates, handoffs, escalations, reviews, and client or stakeholder conversations.
- Turn vague requests into specific questions about evidence, owner, deadline, constraint, risk, and decision rights.
- Push back on unsafe, unsupported, noncompliant, unrealistic, or poorly scoped proposals while preserving professional trust.
- Handle realistic dialogues from the field, including conflict, uncertainty, documentation gaps, customer or stakeholder pressure, and cross-functional disagreement.
- Select language that produces concise workplace outputs: briefing notes, escalation updates, meeting scripts, risk memos, decision records, and follow-up messages.

Instructor Module Plans

Module 1. User Needs and Design Inputs (90 minutes)

Translate user pain points into controlled requirements.

Learners should be able to

- Use these terms accurately: user need, design input, intended use, traceability.
- Explain the workplace tension: User need, design input, intended use, and risk must be separated.
- Respond professionally when a stakeholder says: Add it to the roadmap as requested.
- Select the evidence, tradeoff, owner, and decision required for a design-input clarification.

Customized scenario

Workplace pressure

A surgeon requests a feature after a product demo.

Add it to the roadmap as requested.

User need, design input, intended use, and risk must be separated.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.

4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a design-input clarification.

Module 2. Risk Management and Hazard Analysis (90 minutes)

Discuss hazards, harms, mitigations, and residual risk.

Learners should be able to

- Use these terms accurately: hazard, harm, FMEA, residual risk.
- Explain the workplace tension: Severity, probability, detectability, and mitigation evidence need review.
- Respond professionally when a stakeholder says: Document that the risk is low and move on.
- Select the evidence, tradeoff, owner, and decision required for a risk-control rationale.

Customized scenario

Workplace pressure

A team calls a rare use error acceptable.

Document that the risk is low and move on.

Severity, probability, detectability, and mitigation evidence need review.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a risk-control rationale.

Module 3. Verification, Validation, and Design Review (90 minutes)

Distinguish building the product right from building the right product.

Learners should be able to

- Use these terms accurately: verification, validation, design review, acceptance criteria.
- Explain the workplace tension: Validation, usability, clinical workflow, and traceability may still be open.
- Respond professionally when a stakeholder says: Say yes because verification passed.
- Select the evidence, tradeoff, owner, and decision required for a V&V readiness update.

Customized scenario

Workplace pressure

Leadership asks whether testing is finished.

Say yes because verification passed.

Validation, usability, clinical workflow, and traceability may still be open.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.

3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a V&V readiness update.

Module 4. Usability and Human Factors (90 minutes)

Explain use errors without blaming users.

Learners should be able to

- Use these terms accurately: human factors, use error, formative study, summative study.
- Explain the workplace tension: Interface design, labeling, workflow, and risk controls need assessment.
- Respond professionally when a stakeholder says: Call it a training problem.
- Select the evidence, tradeoff, owner, and decision required for a human-factors finding.

Customized scenario

Workplace pressure

A nurse repeatedly selects the wrong mode during formative testing.

Call it a training problem.

Interface design, labeling, workflow, and risk controls need assessment.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a human-factors finding.

Module 5. Regulatory Pathways and Submissions (90 minutes)

Use pathway language without promising clearance or approval.

Learners should be able to

- Use these terms accurately: 510(k), De Novo, PMA, predicate device.
- Explain the workplace tension: Predicate comparison, indications, performance data, and agency review remain uncertain.
- Respond professionally when a stakeholder says: Tell customers clearance is straightforward.
- Select the evidence, tradeoff, owner, and decision required for a regulatory pathway caveat.

Customized scenario

Workplace pressure

Sales wants to say clearance is guaranteed because predicates exist.

Tell customers clearance is straightforward.

Predicate comparison, indications, performance data, and agency review remain uncertain.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.

2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a regulatory pathway caveat.

Module 6. Complaints, MDRs, and Postmarket Signals (90 minutes)

Triage field reports with safety and regulatory discipline.

Learners should be able to

- Use these terms accurately: complaint, MDR, malfunction, postmarket surveillance.
- Explain the workplace tension: Complaint intake, adverse-event assessment, and reporting timelines may apply.
- Respond professionally when a stakeholder says: Handle it informally with the hospital.
- Select the evidence, tradeoff, owner, and decision required for a complaint intake summary.

Customized scenario

Workplace pressure

A field rep hears about a device malfunction during a case.

Handle it informally with the hospital.

Complaint intake, adverse-event assessment, and reporting timelines may apply.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a complaint intake summary.

Module 7. Manufacturing, Suppliers, and Nonconformance (90 minutes)

Communicate product-quality problems without hiding supply risk.

Learners should be able to

- Use these terms accurately: nonconformance, supplier CAPA, process validation, lot traceability.
- Explain the workplace tension: Nonconformance, disposition, supplier CAPA, and process validation must be resolved.
- Respond professionally when a stakeholder says: Use the parts to avoid backorder.
- Select the evidence, tradeoff, owner, and decision required for a supplier quality update.

Customized scenario

Workplace pressure

A supplier change caused dimensional variation.

Use the parts to avoid backorder.

Nonconformance, disposition, supplier CAPA, and process validation must be resolved.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a supplier quality update.

Module 8. Clinical Training and Labeling Boundaries (90 minutes)

Train users while staying inside cleared indications and instructions.

Learners should be able to

- Use these terms accurately: labeling, instructions for use, off-label, clinical evidence.
- Explain the workplace tension: Labeling, intended use, clinical evidence, and compliance boundaries control the response.
- Respond professionally when a stakeholder says: Demonstrate the technique to build enthusiasm.
- Select the evidence, tradeoff, owner, and decision required for a label-safe training response.

Customized scenario

Workplace pressure

A key opinion leader asks about an off-label technique.

Demonstrate the technique to build enthusiasm.

Labeling, intended use, clinical evidence, and compliance boundaries control the response.

Classroom sequence

1. Terminology selection: distinguish the term that names the decision variable from three plausible alternatives.
2. Risk triage: select the stakeholder, decision, evidence gap, operating constraint, and cost of being wrong from a bounded scenario set.
3. Pushback ladder: choose the strongest sequence from clarification to evidence-based objection to consequence to decision request.
4. Decision artifact check: select the facts, caveat, owner, and next action that belong in a label-safe training response.

Nomenclature and Jargon

These are classroom working definitions. Learners should adapt wording to their organization's policies, systems, and local regulatory environment.

User Needs and Design Inputs

Term	Working meaning	Collocations	Contrast and workplace line
user need	In medical devices, user need is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to translate user pain points into controlled requirements.	clarify user need; document user need	Contrast: user need should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify user need and the evidence that supports it.

Term	Working meaning	Collocations	Contrast and workplace line
design input	In medical devices, design input is a controlled description of how a product, service, system, or built asset is intended to work. Use it when teams need to translate user pain points into controlled requirements.	clarify design input; document design input	Contrast: design input should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify design input and the evidence that supports it.
intended use	In medical devices, intended use is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to translate user pain points into controlled requirements.	clarify intended use; document intended use	Contrast: intended use should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify intended use and the evidence that supports it.
traceability	In medical devices, traceability is information that can be checked to support a claim, decision, investigation, or compliance conclusion. Use it when teams need to translate user pain points into controlled requirements.	clarify traceability; document traceability	Contrast: traceability should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify traceability and the evidence that supports it.

Risk Management and Hazard Analysis

Term	Working meaning	Collocations	Contrast and workplace line
hazard	In medical devices, hazard is a condition or event that can create harm, loss, noncompliance, delay, or unacceptable performance. Use it when teams need to discuss hazards, harms, mitigations, and residual risk.	assess hazard; mitigate hazard	Contrast: hazard identifies a possible or current exposure; it is not the same as accepting that exposure. Example: We need to assess hazard, name the owner, and agree on the trigger for escalation.
harm	In medical devices, harm is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to discuss hazards, harms, mitigations, and residual risk.	clarify harm; document harm	Contrast: harm should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify harm and the evidence that supports it.
FMEA	Failure mode and effects analysis, a structured method for identifying how a process or product can fail and prioritizing controls.	clarify FMEA; document FMEA	Contrast: FMEA should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify FMEA and the evidence that supports it.
residual risk	In medical devices, residual risk is a condition or event that can create harm, loss, noncompliance, delay, or unacceptable performance. Use it when teams need to discuss hazards, harms, mitigations, and residual risk.	assess residual risk; mitigate residual risk	Contrast: residual risk identifies a possible or current exposure; it is not the same as accepting that exposure. Example: We need to assess residual risk, name the owner, and agree on the trigger for escalation.

Verification, Validation, and Design Review

Term	Working meaning	Collocations	Contrast and workplace line
verification	In medical devices, verification is a repeatable control mechanism used to prevent, detect, or confirm conditions before work proceeds. Use it when teams need to distinguish building the product right from building the right product.	clarify verification; document verification	Contrast: verification should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify verification and the evidence that supports it.
validation	In medical devices, validation is a repeatable control mechanism used to prevent, detect, or confirm conditions before work proceeds. Use it when teams need to distinguish building the product right from building the right product.	clarify validation; document validation	Contrast: validation should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify validation and the evidence that supports it.
design review	In medical devices, design review is a structured examination of evidence, process, or results used to reach a defensible conclusion. Use it when teams need to distinguish building the product right from building the right product.	conduct a design review; document the design review	Contrast: design review is a structured examination of evidence; it is not a conclusion reached before the evidence is checked. Example: The team will conduct a design review before it recommends a corrective action.
acceptance criteria	In medical devices, acceptance criteria is an approved rule or decision framework that sets what is permitted, required, or acceptable. Use it when teams need to distinguish building the product right from building the right product.	apply the acceptance criteria; comply with the acceptance criteria	Contrast: acceptance criteria is an approved rule or threshold, not an informal preference. Example: Please confirm which acceptance criteria applies before we commit the team.

Usability and Human Factors

Term	Working meaning	Collocations	Contrast and workplace line
human factors	In medical devices, human factors is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to explain use errors without blaming users.	clarify human factors; document human factors	Contrast: human factors should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify human factors and the evidence that supports it.
use error	In medical devices, use error is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to explain use errors without blaming users.	clarify use error; document use error	Contrast: use error should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify use error and the evidence that supports it.
formative study	In medical devices, formative study is a structured examination of evidence, process, or results used to reach a defensible conclusion. Use it when teams need to explain use errors without blaming users.	clarify formative study; document formative study	Contrast: formative study should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify formative study and the evidence that supports it.
summative study	In medical devices, summative study is a structured examination of evidence, process, or results used to reach a defensible conclusion. Use it when teams need to explain use errors without blaming users.	clarify summative study; document summative study	Contrast: summative study should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify summative study and the evidence that supports it.

Regulatory Pathways and Submissions

Term	Working meaning	Collocations	Contrast and workplace line
510(k)	US FDA premarket submission that typically demonstrates substantial equivalence to a legally marketed predicate device.	prepare a 510(k); identify a predicate device	Contrast: A 510(k) is a regulatory submission pathway, not a general approval for any future device use. Example: Regulatory will prepare the 510(k) around the intended use and predicate comparison.
De Novo	In medical devices, De Novo is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to use pathway language without promising clearance or approval.	clarify De Novo; document De Novo	Contrast: De Novo should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify De Novo and the evidence that supports it.
PMA	In medical devices, PMA is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to use pathway language without promising clearance or approval.	clarify PMA; document PMA	Contrast: PMA should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify PMA and the evidence that supports it.
predicate device	In medical devices, predicate device is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to use pathway language without promising clearance or approval.	clarify predicate device; document predicate device	Contrast: predicate device should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify predicate device and the evidence that supports it.

Complaints, MDRs, and Postmarket Signals

Term	Working meaning	Collocations	Contrast and workplace line
complaint	In medical devices, complaint is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify complaint; document complaint	Contrast: complaint should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify complaint and the evidence that supports it.
MDR	In medical devices, MDR is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify MDR; document MDR	Contrast: MDR should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify MDR and the evidence that supports it.
malfunction	In medical devices, malfunction is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify malfunction; document malfunction	Contrast: malfunction should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify malfunction and the evidence that supports it.
postmarket surveillance	In medical devices, postmarket surveillance is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to triage field reports with safety and regulatory discipline.	clarify postmarket surveillance; document postmarket surveillance	Contrast: postmarket surveillance should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify postmarket surveillance and the evidence that supports it.

Manufacturing, Suppliers, and Nonconformance

Term	Working meaning	Collocations	Contrast and workplace line
nonconformance	Failure to meet a requirement, specification, contract, standard, or approved procedure.	clarify nonconformance; document nonconformance	Contrast: nonconformance should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify nonconformance and the evidence that supports it.
supplier CAPA	In medical devices, supplier CAPA is a stakeholder category whose needs, eligibility, experience, or obligations affect the decision. Use it when teams need to communicate product-quality problems without hiding supply risk.	clarify supplier CAPA; document supplier CAPA	Contrast: supplier CAPA should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify supplier CAPA and the evidence that supports it.
process validation	In medical devices, process validation is a repeatable control mechanism used to prevent, detect, or confirm conditions before work proceeds. Use it when teams need to communicate product-quality problems without hiding supply risk.	clarify process validation; document process validation	Contrast: process validation should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify process validation and the evidence that supports it.
lot traceability	Ability to follow a production lot through materials, process steps, tools, inspections, and downstream customers or products.	clarify lot traceability; document lot traceability	Contrast: lot traceability should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify lot traceability and the evidence that supports it.

Clinical Training and Labeling Boundaries

Term	Working meaning	Collocations	Contrast and workplace line
labeling	In medical devices, labeling is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify labeling; document labeling	Contrast: labeling should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify labeling and the evidence that supports it.
instructions for use	In medical devices, instructions for use is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify instructions for use; document instructions for use	Contrast: instructions for use should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify instructions for use and the evidence that supports it.
off-label	In medical devices, off-label is a field-specific concept used to locate the relevant fact, boundary, or decision variable. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify off-label; document off-label	Contrast: off-label should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify off-label and the evidence that supports it.
clinical evidence	In medical devices, clinical evidence is information that can be checked to support a claim, decision, investigation, or compliance conclusion. Use it when teams need to train users while staying inside cleared indications and instructions.	clarify clinical evidence; document clinical evidence	Contrast: clinical evidence should be defined against the relevant evidence and decision boundary, rather than used as a vague label. Example: Before we proceed, please clarify clinical evidence and the evidence that supports it.

Industry-Specific Meeting Moves

Situation	Useful language
User Needs and Design Inputs	Before we commit, I want to confirm user need, design input, the owner, and the evidence behind the decision. If user need, design input, intended use, and risk must be separated., I recommend we document the risk and agree on the next step.
Risk Management and Hazard Analysis	Before we commit, I want to confirm hazard, harm, the owner, and the evidence behind the decision. If severity, probability, detectability, and mitigation evidence need review., I recommend we document the risk and agree on the next step.
Verification, Validation, and Design Review	Before we commit, I want to confirm verification, validation, the owner, and the evidence behind the decision. If validation, usability, clinical workflow, and traceability may still be open., I recommend we document the risk and agree on the next step.
Usability and Human Factors	Before we commit, I want to confirm human factors, use error, the owner, and the evidence behind the decision. If interface design, labeling, workflow, and risk controls need assessment., I recommend we document the risk and agree on the next step.
Regulatory Pathways and Submissions	Before we commit, I want to confirm 510(k), De Novo, the owner, and the evidence behind the decision. If predicate comparison, indications, performance data, and agency review remain uncertain., I recommend we document the risk and agree on the next step.
Complaints, MDRs, and Postmarket Signals	Before we commit, I want to confirm complaint, MDR, the owner, and the evidence behind the decision. If complaint intake, adverse-event assessment, and reporting timelines may apply., I recommend we document the risk and agree on the next step.
Manufacturing, Suppliers, and Nonconformance	Before we commit, I want to confirm nonconformance, supplier CAPA, the owner, and the evidence behind the decision. If nonconformance, disposition, supplier capa, and process validation must be resolved., I recommend we document the risk and agree on the next step.
Clinical Training and Labeling Boundaries	Before we commit, I want to confirm labeling, instructions for use, the owner, and the evidence behind the decision. If labeling, intended use, clinical evidence, and compliance boundaries control the response., I recommend we document the risk and agree on the next step.

High-pressure pushback frames

- I understand the urgency. The risk is that we move faster than the evidence or process supports.
- I am not blocking the goal. I am naming the condition we need before the decision is safe and credible.
- If we accept this risk, we should name the owner, document the assumption, and define the trigger for escalation.
- That may be possible, but not under the current scope, timeline, or approval path.
- Let's separate what we know, what we assume, and what still needs confirmation.

Assessment and Coaching

Performance rubric

Skill	Developing	Proficient	Strong
Terminology	Recognizes terms but uses them loosely.	Uses field terms accurately in context.	Defines terms, connects them to evidence, and explains decision impact.
Pushback	Disagrees vaguely or avoids disagreement.	Names concern with evidence and next step.	Balances urgency, relationship, risk, owner, and decision rights.
Scenario judgment	Focuses on one stakeholder's preference.	Identifies constraint, risk, and process.	Guides the group toward a documented, realistic decision.
Decision communication	Selects language without linking it to the decision.	Chooses language that names facts, owner, and next step.	Distinguishes evidence, tradeoff, authority, and escalation in a decision-ready response.

Source orientation

- Current FDA medical device and quality-system resources.
- Company design-control, complaint, and regulatory procedures.
- Approved labeling and instructions for use.
- The learner's own company policies, SOPs, contracts, systems, templates, and approved communication standards.